

Boil (Skin Abscess) - Female (Child)

Algorithm 1140 | Child (0-17 years) | Gender at birth: Female

Definition

- Boil/skin abscess assessment decomposed from NHS.UK's published when-to-get-help guidance.
- Child patient.
- Female.

Initial assessment

1. [LOCATION] Where is the boil?
2. [DURATION] How long has it been there?
3. [YES_NO] Does the caller have diabetes or a weakened immune system?
4. [CAREGIVER_OBSERVATION] What change has a safe parent or caregiver observed in alertness, breathing, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Disposition level 50

Is a widespread or flat localized rash the main concern, without a painful pus-filled lump or abscess?

The localized-rash family is not generated. Arrange in-person clinical assessment through the Qatar route approved by governance (GOVERNANCE_REQUIRED).

1. Disposition level 100

Is there confusion, collapse, abnormal breathing, pale/blue/mottled skin, rapidly spreading redness or swelling, severe pain out of proportion, black skin, or swelling around the eye, nose, jaw, or neck affecting vision, swallowing, or breathing?

Sepsis, necrotising infection, orbital spread, and airway involvement require immediate emergency assessment.

1. Disposition level 70

Without an emergency feature, is the boil on the face, near the eye, genitals, breast, spine, hand, or over a joint; is redness spreading or fever present; or is the patient an infant, pregnant, diabetic, immunocompromised, or otherwise seriously unwell?

NHS.UK boils guidance lists these as reasons for an urgent clinical review.

1. Disposition level 50

Has the boil lasted 2 weeks without improving, does the caller keep getting boils, or is there a cluster of boils (carbuncle)?

NHS.UK guidance recommends a routine primary-care review for a persistent boil, recurring boils, or a carbuncle.

1. Disposition level 15

Is this a single, recent boil with none of the features above?

NHS.UK guidance provides self-care steps (warm compresses) for an uncomplicated boil.

Advice

Emergency skin-infection precautions

- Call Qatar 999 now. Do not squeeze, lance, cut, or apply caustic substances. Keep the patient still, do not allow self-driving, and follow the call-handler's instructions. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: confusion, collapse, breathing change, rapidly spreading swelling, black skin, or severe pain.

Prompt in-person abscess review

- Use the Qatar governance-approved in-person service. Do not squeeze or pierce the lesion or use leftover antibiotics. Pregnancy, infant, diabetes, immune suppression, facial and genital lesions require clinician-led treatment.
- Call Back If: spreading redness, fever, severe pain, or swelling near eye or airway.

Routine boil follow-up

- Arrange a routine primary-care review for a persistent, recurring, or clustered boil.
- Call Back If: boil worsens; fever develops.

Care for a localized boil pending Qatar review

- Use a clean warm compress for short periods and keep the area clean and covered if it drains. Do not pick, squeeze, pierce, or apply caustic products, and do not share towels. Medicine choice and dose require age, weight, pregnancy or breastfeeding status, allergies, kidney or liver disease, ulcer or bleeding risk, other medicines, and Qatar formulary approval. This UAT branch remains non-telemedicine pending clinical governance.

- Call Back If: boil lasts more than 2 weeks; signs of spreading infection or fever; pain or swelling rapidly worsens.

First aid

- Call Qatar 999 now. Do not squeeze, lance, cut, or apply caustic substances. Keep the patient still, do not allow self-driving, and follow the call-handler's instructions. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Boils", <https://www.nhs.uk/conditions/boils/> (page last reviewed 20 June 2023)

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from the cited public sources; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect and is not a disposition row. Demographic variants contain only source-supported differences; identical thresholds are explicitly documented rather than fabricated. Local destination mappings and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

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